



IGF1-LR3 RESEARCH HUB · RESEARCH USE ONLY

BUILD MUSCLE. RECOVER FASTER.

FOR MEN & WOMEN. Build muscle, burn fat, and recover faster.

[Get IGF-1 LR3 1 mg at BioLinx](#)

[Read the guide first](#)

Also shown to help with: muscle growth · fat loss · insulin sensitivity
Daily · around your workout, or morning · Difficulty: beginner

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RESEARCH SHOWED THAT...

30 hrs active
vs. 10 minutes for regular IGF-1.

Maple & Monis, 2024

It proved 3× more potent than standard IGF-1 due to two key structural modifications.

Maple & Monis, 2024



STEP 1 OF 4 · WHAT TO BUY

SHOPPING LIST

Easy-to-follow shopping list to safely practice IGF1-LR3. Match your commitment level — don't skip anything.



PEPTIDE · ONE ORDER

IGF-1 LR3 1 MG

Build muscle, burn fat, and recover faster.
Common starting dose: **0.02 mg**, daily,
around your workout, or morning.

Get IGF-1 LR3 1 mg at BioLinx

biolinxlabs.com/products/igf-1-lr3-1-mg

HOW MANY VIALS TO BUY · BEST VALUE BY GOAL

GOAL	VIAL	QTY	WHERE
1 MONTH	1 mg vial	1×	<u>See it</u> biolinxlabs.com/products/igf-1-lr3-1-mg
FULL CYCLE RECOMMENDED	1 mg vial	2×	<u>See it</u> biolinxlabs.com/products/igf-1-lr3-1-mg

MATERIALS ≈ \$41 ON TOP — FULL LIST ON THE NEXT PAGE



PRODUCTS ARRIVED?

QUICK SUMMARY OF YOUR LAB ITEMS:

TWO VIALS IN THE BOX • IGF-1 LR3 1 MG + BACTERIOSTATIC WATER 10 ML

VIAL 1 **IGF-1 LR3 1 mg.** Arrives as a powder. Don't touch it until you've read page 4.

VIAL 2 **Bacteriostatic water — 10 mL.** This is what turns the powder into a liquid. Sterility and pH matter — don't substitute.

GROCERY LIST • ≈ \$41

ITEM	SPEC	QTY	WHERE
INSULIN SYRINGES BUY	1cc U100, 30G, 5/16"	50–100 ct	Any pharmacy
ALCOHOL PREP PADS BUY	Any brand, 70% isopropyl	50–100 ct	Any pharmacy
SHARPS CONTAINER BUY	1 quart, for needle disposal	1×	Any pharmacy
BAC WATER BUY	Bacteriostatic water, 10 mL — sterility and pH matter, don't substitute	1 vial	<u>See it</u> biolinxlabs.com/products/bacteriostatic-water-10-ml

Any brand works for syringes, pads and sharps — but not BAC water. Use 2 fresh syringes and 2 fresh pads every session: one to mix, one to inject.

HOW TO STORE IT

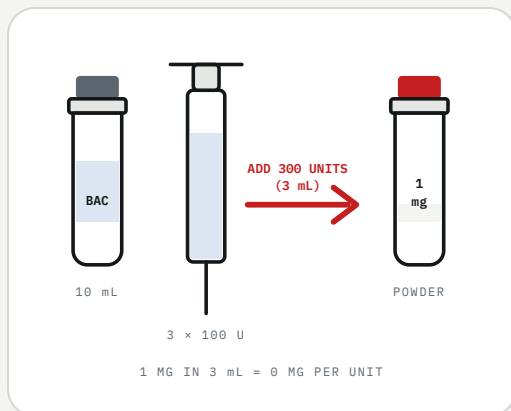
POWDER	Freezer — lasts years
RECONSTITUTED	Fridge — about 28 days. Write the mix date on the vial.
WATER	Room temperature — check the expiration date



STEP 2 OF 4 · RECONSTITUTION – FROM POWDER TO LIQUID

LET'S “FORM” YOUR PEPTIDE!

“Form” means “reconstitute”. Your product arrives as a powder — you need to liquify it before “you begin your research.” 100 units = one full syringe. **1 mg vial** → **add 300 units (3 mL)**. Every dose in this guide assumes exactly that ratio, so measure it — don't eyeball it.



1 Remove the plastic caps & wipe both rubber vial tops

Fresh alcohol pad on the IGF1-LR3 vial and the bac water vial.

2 Draw 300 units of bac water

That's 3 full pulls on a 100-unit insulin syringe (3 mL total).

3 Inject it slowly down the side of the 1 mg vial

Let it run down the glass. Don't blast the powder.

4 Swirl gently. Never shake. Place it in the fridge!

Slow and gentle wins this one — shaking wastes peptide. Discard that syringe into the sharps container. Let it rest until completely clear (36–46 °F); use within ~28 days; never freeze it.

YOUR SIMPLIFIED UNITS

DOSE	FILL TO
0.02 mg	6 units
0.03 mg	9 units
0.04 mg	12 units
0.05 mg	15 units
0.06 mg	18 units
0.08 mg	24 units
0.1 mg	30 units

Change the vial, water or dose →
[interactive calculator](#)



STEP 3 OF 4 · INJECT

WHERE TO INJECT

Time to inject · clean, pinch, poke, done. Daily, around your workout, or morning.

1

Take IGF1-LR3 out of the refrigerator and extract your dose

Your first dose is **0.02 mg = 6 units** on a 100-unit syringe. Wipe the vial top with a fresh alcohol pad, let it air-dry a few seconds, then draw your units.

2

Pinch a fold of skin on the stomach + use an alcohol wipe

Two inches away from the belly button. Wipe with a fresh alcohol pad, let it air-dry, and don't touch that spot again before injecting. Rotate sites.

3

Needle in at the base of the fold

Push the plunger slowly and steadily. Hold for a few seconds, then withdraw.

4

That's it — you just did your first shot!

Drop the needle straight into the sharps bin. Vial goes back in the fridge.



STOMACH · PINCH A FOLD · NEEDLE AT THE BASE

WATCH FIRST

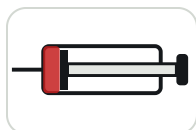
Mixing and injecting from start to finish, on video: professorpeptides.com. Use 2 fresh syringes and 2 fresh pads every session — one to mix, one to inject.



STEP 4 OF 4 · DOSING & TIMING SCHEDULE

IGF1-LR3 SCHEDULE (PROTOCOL)

“Schedule” means “protocol”. Everything below assumes the 1 mg vial was reconstituted with exactly 300 units (3 mL), read off a standard 100-unit insulin syringe.



FIRST DOSE

How much, how often & when?

WHEN

Around your workout, or morning. Daily.

HOW MUCH

0.02 mg to start.

Dose runs in on/off blocks with breaks between — this is **cycling**, not titration. Full schedule on page 9.

UNITS

Extract **6 units** on a 100-unit insulin syringe.

6 units = 0.02 mg. Exact variables: 100-unit syringe · 1 mg vial reconstituted with 300 units · 0 mg per unit.

HOW OFTEN

Daily. Hold each step for its full block before moving up.

CYCLE

Stay within 0.02–0.05 mg to minimize risk · 1 month on, 2 months off.

Stay within the 0.02–0.05 mg range to minimize risk. Don't rush it — start at the lowest amount, hold each step for its full block, and only step up if the current dose is comfortable. Most people never need the top doses.



STEP 4 OF 4 · DOCUMENT

WHAT TO EXPECT + SIDE EFFECTS

Mostly mild, mostly early, mostly right after a dose increase. Know the three tiers before the first shot, not after.

NORMAL / EXPECTED

First few days

- Mild headache or muscle/joint soreness in the first week
- Redness at the injection site
- Feeling hungry or slightly tired after a dose

COMMON, WORTH NOTING

Manage it, don't push the dose

- Low blood sugar 30–60 minutes after a shot — shakiness, sweating, hunger; eat 30 minutes before injecting
- Water retention or joint stiffness at higher doses
- Growth-factor effects build with time — stay in the 0.02–0.05 mg range

STOP & CALL A DOCTOR

Don't push through it

- Confusion, severe shakiness or fainting (serious low blood sugar) — eat sugar and get help
- Signs of an allergic reaction — swelling, rash, difficulty breathing
- Any new lump, or a fast-growing skin change

NOTE: for this one it is **HIGHLY RECOMMENDED** to start at the lowest amount first and work up over time, as needed. IGF-1 drives cell growth: not for anyone with an active cancer or a history of cancer, diabetes, or who is pregnant — talk to a doctor before use. Never inject on an empty stomach.



FIRST-TIME TL;DR

RECAP: CHECKLIST

Everything from pages 2–7, in the order it happens.

BEFORE

- ☐ Vial reconstituted with exactly 300 units of bac water, resting in the fridge, mix date written on it
- ☐ Supplies laid out: new syringe, alcohol pad, sharps container
- ☐ Dose set at **0.02 mg = 6 units** — not higher, no matter what a forum says

DURING

- ☐ Inject daily, around your workout, or morning — stomach, rotate the spot
- ☐ Note the date and dose somewhere — a phone note is fine

AFTER

- ☐ Expect possible redness, mild irritation or a headache — it settles
- ☐ Mild nausea can happen after a dose increase — sit tight, it typically passes
- ☐ If anything feels more than mild — dizziness, real nausea, allergic reaction — stop and get medical advice

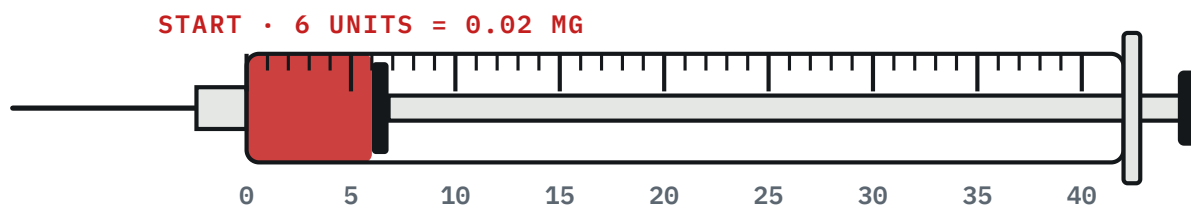
NEXT BLOCK

- ☐ Hold each step for its full block before stepping up — never skip a step (stay within 0.02–0.05 mg to minimize risk · 1 month on, 2 months off)
 - ☐ Only step up if the current dose is tolerated; if side effects are rough, stay another block
 - ☐ Fresh syringe, fresh pad, needle in the sharps bin — every single time
-

UNIT CHEAT-SHEET · 100-UNIT SYRINGE · 1 MG VIAL RECONSTITUTED WITH 300 UNITS

HOW TO CYCLE (RAMP-UP & CYCLE SCHEDULE)

Assumes the 1 mg vial was mixed with exactly 300 units, so **1 unit = 0 mg**. Units shown are per shot.



WHEN	DOSE	SYRINGE · PER SHOT	NOTES
Month 1	0.02 mg/day · 0.1 mg/week	6 units	START HERE
Months 2–3	Off	—	REST
Month 4	0.05 mg/day · 0.25 mg/week	15 units	MAX DOSE
Months 5–6	Off	—	REST

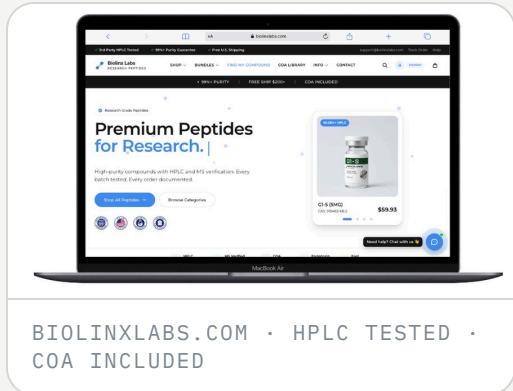
Rule of thumb: don't move up a step until you know how the current dose feels. Only step up if the previous block was tolerated — Stay within the 0.02–0.05 mg range to minimize risk. Most people never need to go past the middle of the ramp.



THE IGF1-LR3 SOURCING GUIDE

PROFESSOR PEPS: SOURCING GUIDE (APPENDIX)

How to verify any supplier in 90 seconds. A legitimate source clears all three checks below in under two minutes — no back-and-forth required.



1

Ask for the COA

A batch-specific Certificate of Analysis — not a generic PDF reused across batches — showing purity $\geq 98\%$ via HPLC/MS.

2

Match the lot number on the vial

It must match the lot number on the COA. If they don't match, walk away.

3

Check the track record

Searchable reviews or forum mentions older than roughly six months. Brand-new storefronts with no history are the highest-risk category.

DON'T WORRY — WE'VE ALREADY DONE THE VETTING

BioLinx Labs clears the list: third-party HPLC testing, a 99%+ purity guarantee and a COA included with every order. biolinxlabs.com/best-sellers

biolinxlabs.com/best-sellers

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**STAY CURIOUS,
PROFESSOR PEPTIDES**

IGF1-LR3 Research Hub · companion to the email series